OUTREACH FOLLOW-UP NOTE · DAURA LGA, KATSINA STATE

Who came, who stayed home, and what to do next

The people who came and the people who stayed home are held back by different things. Listening only to those who came would point the in-charge to the wrong village and the wrong fix.

30
people who attended (Kusugu and Sarkin Yara)

40
households that did not attend, asked at home

3
outreach staff who shared their view

What the numbers say

1. **The information gap is in a different village than attendees suggest.** Among attendees, Kusugu named “lack of information” most (5 of 15, against 2 of 15 in Sarkin Yara). Among households that stayed home, it is Sarkin Yara: 8 of 20 never heard about the outreach, against 4 of 20 in Kusugu.
2. **Most who missed it still want to come.** Of the 12 households that never heard, 10 say Yes or Maybe to attending next time. Across all 40 households, 34 say Yes or Maybe. For these families, the problem was not willingness.
3. **The two groups ask for different things.** Those who came want shorter waits. Those who stayed home live farther away and want something closer, a reminder, or a visit.

	People who came (30)	Households that stayed home (40)
Biggest ask	Shorter waiting time: 10 of 29 who answered	Closer service point: 9 of 40. More information: 8. Reminder call or SMS: 6. Home visit: 5
Distance and travel	Typical: 2.3 km to the outreach center, 33 minutes travel time	Typical: 3.7 km to the outreach center, 45 minutes travel time
Waiting	7 of 29 who answered waited over an hour, and they gave the lowest satisfaction scores	2 of 40 stayed home because they heard waits were long
What worked for reach	Half heard through a community health worker or community leader	Of the 28 who did hear, 17 (61%) heard through a community health worker or community leader
Would come (again)	Yes 18, Maybe 9, No 3	Yes 20, Maybe 14, No 6

What the outreach team said

Three outreach staff shared their view, all from Kusugu. None were recorded in Sarkin Yara, where the gap is biggest, so that team still needs to be heard. They rated community mobilisation 2 out of 5 on average, which matches households saying they never heard or heard too late (17 of 40). Two of the three team members named transport as their main challenge, and when asked what to improve first, two chose more supplies, and one chose earlier mobilisation. With only three records, treat this as a hint, not a finding.

Three actions for the in-charge

1. Start with Sarkin Yara, and use the channel that already works. Ask the community health worker and the community leader there to tell households directly about the date and place, and to send a reminder (call or message) the day before.
Why: 8 of 20 households there never heard, and word from health workers and leaders reached most of those who did.
Progress check: Next time, ask arrivals how they heard, and count Sarkin Yara arrivals against this time to see whether it improved.

2. Find where the delay happens, then cut it. On outreach day, give each arrival a time slip at registration and record when they are seen. Review it with the team afterwards: if most of the wait is before registration is done, consider adding a second registration point; if it is after, consider adding staff or starting earlier.
Why: Shorter waiting time was attendees’ top request (10 of 29), and 7 of 29 attendees with a recorded wait (24%) waited over an hour and rated the visit lowest (about 2.9 out of 5, against about 3.2 for shorter waits).
Progress check: Next time, compare waiting times with the previous session and assess whether fewer people waited over an hour.

3. Bring the service closer to those farthest away. For households beyond about 3 km, plan one extra session day at a nearer point, or a home visit for those who cannot travel.
Why: Households that stayed home live farther away, and 14 of 40 (35%) asked for a closer point or a visit. Two of the 3 staff also named transport as their main challenge.
Progress check: Next time, review the number of households reached from beyond 3 km.

Limits. This page uses practice data, not real households, so its patterns are illustrative and not findings about Daura. The data shows what goes together, not what causes what.

Sharon Imaledo · Data analyst